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- (ii) Identify missing data, inconsistent data, data outliers, unexpected lack of variability, and protocol deviations.
 - (iii) Examine data trends, such as the range, consistency, and variability of data within and across sites.
 - (c) Identifying significant errors in data collection and reporting at a site or across sites, potential data manipulation, and data integrity problems.
- vi. Monitoring report (3.11.4.6)
- (a) Reports of monitoring activities should include a summary of what was reviewed, a description of significant findings, conclusions and actions required to resolve them, and follow-up on their resolution including those not resolved in previous reports. The requirements of monitoring reports (including their content and frequency) should be described in the sponsor's procedures.
 - (b) Reports of investigator site and/or centralized monitoring should be provided to the appropriate sponsor staff as described in the sponsor's procedures in a timely manner for review and follow-up.
 - (c) When needed, the report should describe findings requiring escalation for action and resolution. The sponsor should decide on the appropriate action to be taken, and these decisions and the resolution of the actions involved, where needed, should be recorded.

12. Noncompliance (3.12)

- 3.12.1 Noncompliance with the protocol, SOPs, GCP, and/or applicable regulatory requirement(s) by an investigator/institution or by member(s) of the sponsor's staff should lead to appropriate and proportionate action by the sponsor to secure compliance.
- 3.12.2 If noncompliance that significantly affects or has the potential to significantly affect the rights, safety or well-being of trial participant(s) or the reliability of trial results is discovered, the sponsor should perform a root cause analysis, implement appropriate corrective and preventive actions, and confirm their adequacy unless otherwise justified. Where the sponsor identifies issues that are likely to significantly impact the rights, safety or well-being of the trial participant(s), or the reliability of trial results (i.e., serious noncompliance), the sponsor should notify the regulatory authority and/or IRB/IEC, in accordance with applicable regulatory requirements, and/or investigator, as appropriate.
- 3.12.3 If significant noncompliance is identified on the part of an investigator/institution or service provider that persists despite efforts at remediation, the sponsor should consider terminating the investigator's/institution's or service provider's participation in the trial. In these circumstances, the sponsor should promptly notify the regulatory authority(ies) and IRB/IEC of the serious noncompliance, as appropriate, and take

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actions to minimize the impact on the trial participants and the reliability of the results.

13. Safety Assessment and Reporting (3.13)

The sponsor is responsible for the ongoing safety evaluation of the investigational product(s). The Investigator's Brochure or, where applicable, the current scientific information such as a basic product information brochure, forms the basis of safety assessment and reporting for the clinical trial. For further information, see Appendix A.

a. Sponsor review of safety information (3.13.1)

The sponsor should aggregate, as appropriate, and review in a timely manner relevant safety information. This includes the review of any reported unfavorable medical events occurring in participants before investigational product administration (e.g., during screening). This may result in the update of the protocol, Investigator's Brochure, informed consent materials, and related documents.

The sponsor should review the available emerging safety information to assess whether there is any new data that may affect the participant's willingness to continue in the trial, impact the conduct of the trial, or alter the approval/favorable opinion of the IRB/IEC and/or regulatory authority(ies), as applicable. Any information of this nature should be communicated to the participants, investigator, IRB/IEC, and regulatory authorities, as applicable, in a timely manner.

b. Safety reporting (3.13.2)

- (a) The sponsor should submit to the regulatory authority(ies) safety updates and periodic reports, including changes to the Investigator's Brochure, as required by applicable regulatory requirements.
- (b) The sponsor should, in accordance with the applicable regulatory requirement(s) and with ICH guidance for industry *E2A Clinical Safety Data Management: Definitions and Standards for Expedited Reporting* (March 1995), expedite the reporting to the regulatory authority(ies) of all suspected, unexpected and serious adverse reactions (i.e., SUSARs).
- (c) Safety reporting to regulatory authorities should be undertaken by assessing the expectedness of the reaction in relation to the applicable product information (e.g., the reference safety information (RSI) contained within the Investigator's Brochure or alternative documents) in accordance with applicable regulatory requirements. Refer to ICH guidance for industry *E2F Development Safety Update Report* (August 2011) for more information about RSI.
- (d) The reporting of SUSARs to investigator(s)/institutions(s) and to the IRB(s)/IEC(s) should be undertaken in a manner that reflects the urgency of action required and should take into consideration the evolving knowledge of the safety profile of the product and should be performed in accordance with applicable regulatory requirements. In some regions, periodic reporting of line listings with an overall safety assessment may be appropriate.

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(w) The sponsor should have processes and procedures in place for reporting to relevant parties, including regulatory authorities, incidents (including security breaches) that have a significant impact on the trial data.

(x) When using computerized systems in a clinical trial, the sponsor should:

For systems deployed by the sponsor:

- (i) Have a record of the important computerized systems used in a clinical trial. This should include the use, functionality, interfaces, and validation status of each computerized system, and who is responsible for its management should be described. The record should also include a description of implemented access controls and internal and external security measures.
- (ii) Ensure that the requirements for computerized systems (e.g., requirements for validation, audit trails, user management, backup, disaster recovery, and IT security) are addressed and implemented and that documented procedures and adequate training are in place to ensure the correct development, maintenance, and use of computerized systems in clinical trials (see section III.D (4)). These requirements should be proportionate to the importance of the computerized system and the data or activities they are expected to process.
- (iii) Maintain a record of the individual users who are authorized to access the system, their roles, and their access permissions.
- (iv) Ensure that access permissions granted to investigator site staff are in accordance with delegations by the investigator and visible to the investigator.
- (v) Ensure that there is a process in place for service providers and investigators to inform the sponsor of system defects identified.

For systems used or deployed by the investigator/institution:

- (vi) Assess whether such systems, if identified as containing source records in the trial, (e.g., electronic health records, other record keeping systems for source data collection and investigator site files) are fit for purpose or whether the risks from a known issue(s) can be appropriately mitigated. This assessment should occur during the process of selecting clinical trial sites and should be documented.
- (vii) In situations where clinical practice computerized systems are being considered for use in clinical trials (e.g., electronic health records or imaging systems used or deployed by the investigator/institution), these systems should be assessed for their fitness for purpose in the context of the trial.
- (viii) The assessment should be performed before being used in the trial and should be proportionate to the importance of the data managed in the system. Factors such as data security (including measures for backup), user management, and audit trails, which help ensure the protection of confidentiality and integrity of the trial data, should be considered as appropriate.